

Name : Mr. MAULIK GHANSHYAMBHAI THUMMAR 25044964

Age : 25Yr

Gender : Male

UHID : DGRL.0000587981 / GANCWS2050

W/BNo/RefNo : PUP CONTINENTAL
HOSPITALS

Lab No : APJ1.H2610705 LRN : 13244316

Ref Doctor : DR DHANANJAYA K L



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TRANSPLANT KIDNEY BIOPSY (HISTOPATHOLOGY AND IHC SINGLE ANTIGEN)

Brief Clinical History:

Post kidney transplant (07/01/26), live related donor, earlier biopsy - 21/01/26 - acute TCMR.
25/02/26 - mild acute tubular injury, no rejection, urine culture 24/01/26 - E coli, complaints of
nocturia

Serum creatinine 1.75 to 1.9 mg/dL, BKV PCR not detected

Suspected rejection

Specimen:

Allograft kidney (needle) biopsy

Macroscopic Description:

1 core measuring 0.6 cm in length for LM study. Entire tissue processed.

Microscopic Description:

- Serial sections studied show a core of renal cortex with up to seven glomeruli and no larger artefines - 2 glomeruli show ischaemic changes with periglomerular fibrosis. The viable glomeruli appear normal. There is no glomerulitis, increase in cellularity or thrombi.
- Tubules show evidence of injury with mild to moderate tubulitis with an occasional atrophic tubule

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showing severe tubulitis.

Tubular atrophy is seen involving 5%-10% of the cortex sampled.

- Interstitium is widened and oedematous and shows a moderately dense infiltrate of lymphocytes and plasma cells involving 30%-35% of the cortex sampled.

- 1 small artery sampled appears unremarkable. There is no evidence of endarteritis, intimal fibrosis or arteriolar hyalinosis. Occasional peritubular capillary shows mild peritubular capillaritis but not exceeding 10% of the cortex sampled.

Immunohistochemistry Report :

C4d stain shows no positivity in the PTC walls or along the GBM.

SV40 IHC is negative.

Diagnosis/Comments:

- Allograft kidney (needle) biopsy shows moderate tubulitis with interstitial inflammation, consistent with a T - cell mediated rejection, type I A.

- There is mild IF/TA (10%).

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* END OF REPORT *

Reported By: Dr. Meenakshi Swain ., Senior
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